Post-Op Supplement Analysis: FUE Recovery Days 1-21

Approximately 6,300 grafts | Dr. Sean Behnam, Los Angeles | Procedure August 19-20, 2026

All supplement doses verified against manufacturer labels. Concurrent medications: aspirin, pentoxifylline, cephalexin, prednisone (days 1-7); oral minoxidil (half dose); Trintellix 20 mg, olmesartan 20 mg, rosuvastatin 10 mg.

Days 1-3 — Hemostasis and plasmatic imbibition

Grafts are avascular, surviving on plasma diffusion under a fibrin clot. **Needs:** stable clot, low edema, hypoxia tolerance. **Donor:** clot formation, epithelialization begins.

Supplement (confirmed dose)	Harmful	Neutral	Helpful
NAC 1,200 mg	Mild thiol antiplatelet effect. Smallest contributor on your stack.	None	Glutathione precursor. Buffers hypoxic oxidative stress in avascular grafts.
Creatine 5 g	None. Prior edema claim was wrong: creatine water is intracellular and muscle-localized, and oral minoxidil dwarfs it.	Primary MOA	Phosphocreatine ATP buffering in hypoxic tissue (extrapolated, no graft data).
Glycine 3 g	None	Glycine-receptor sleep effects	Collagen substrate. Cytoprotective in ischemia models.
Collagen 14 g + HA 120 mg + vit C **Vital Proteins Advanced**	None	Substrate not yet rate-limiting	Amino pool for early matrix. HA supports provisional matrix hydration.
Urolithin A 1,000 mg **Mitopure, 2x label dose**	None established	Mitophagy induction operates over weeks, not 72 hours	Mitochondrial QC in hypoxic grafts (theoretical).
Ca-AKG 1,000 mg **ProHealth, 2 x 500 mg**	None	Whether oral AKG raises tissue AKG meaningfully is unestablished	Krebs intermediate. Modest energy substrate.
ADAM multi (2 softgels) **NOW Foods**	Vit E 100 mg d-alpha plus grape seed 25 mg. The only real supplement bleed contributors you have.	Most B vitamins and trace minerals at repletion	Vit A 3,000 mcg offsets prednisone-impaired healing. Vit C 250 mg. Zinc 15 mg. K1 35 + K2 45 mcg support clotting.
CoQ10 200 mg	Antiplatelet evidence is thin and mixed. Previously overstated.	Offsets rosuvastatin CoQ10 depletion	Electron transport efficiency under hypoxia.
NAD+ CR + Resveratrol Elite **300 mg NR, 10 mg trans-res, 5 mg quercetin, fisetin**	None. Polyphenol doses too small to matter.	NR raises NAD+ over weeks, not days	ATP and DNA repair support in stressed cells.
Black Maca 2,000 mg	None known	No relevant wound-healing MOA	None
Boron 3 mg	Small free-T/DHT bump, now unopposed with Problend paused. Donor follicles are DHT-resistant; native recipient-area hair is not. Still trivial at 3 mg.	Main MOA	None. Anti-inflammatory effect moot vs prednisone.
Omega-3 850 EPA / 540 DHA **Thorne Super EPA, 2 gelcaps**	None. 1,390 mg is roughly half the threshold for clinically detectable antiplatelet effect.	Main position this phase	None yet
D3/K2 (1,000 IU / 45 mcg) **NOW Foods**	None	D3 effects are repletion-dependent, not acute	K2 supports gamma-carboxylation of clotting factors.

Days 4-7 — Inosculation and revascularization

Graft vessels reconnect to recipient capillaries around day 3-5, followed by a reperfusion ROS burst. **Needs:** angiogenesis, oxidative buffering, infection control. **Donor:** re-epithelialization completes.

Supplement (confirmed dose)	Harmful	Neutral	Helpful
NAC 1,200 mg	Residual, tapering	None	Best-timed benefit of the whole stack: buffers reperfusion ROS at inosculation.
Creatine 5 g	None	Mostly	ATP support during metabolic reactivation.
Glycine 3 g	None	None	Substrate as fibroblasts activate.
Collagen 14 g + HA + vit C	None	None	Proline/hydroxyproline for granulation tissue.
Urolithin A 1,000 mg	None	None	Mitochondrial QC during the reperfusion window.
Ca-AKG 1,000 mg	None	None	Prolyl hydroxylase co-substrate as collagen synthesis initiates.
ADAM multi	Vit E and grape seed still contributing while aspirin and pentoxifylline continue.	Most components	Vit A, C, zinc for donor-site epithelialization. Selenium 200 mcg antioxidant.
CoQ10 200 mg	Negligible	Mostly	Reperfusion antioxidant plus mitochondrial support.
NAD+ CR + Resveratrol	None	None	Fuels proliferating endothelium during revascularization.
Black Maca	None	Main MOA	None
Boron 3 mg	Same trivial androgen note	Main MOA	None
Omega-3	None	Weak theoretical anti-angiogenic signal, not clinically established at this dose	None yet
D3/K2	None	None	D3 drives cathelicidin, relevant while cephalexin is running.

Days 8-14 — Proliferation

Collagen III deposition. Grafts mechanically secure by day 10-14. Shock shedding begins. **Needs:** collagen substrates, vitamin C, zinc, protein, resolving inflammation.

Supplement (confirmed dose)	Harmful	Neutral	Helpful
NAC 1,200 mg	None	None	Cysteine feeds keratin and glutathione synthesis in proliferating keratinocytes.
Creatine 5 g	None	Mostly	Energy for high-turnover tissue.
Glycine 3 g	None	None	One-third of collagen residues. Plausibly rate-relevant at this volume of synthesis.
Collagen 14 g + HA + vit C	None	Substrate may not be limiting if protein intake is adequate	Peak utility phase: direct substrate for collagen III.
Urolithin A 1,000 mg	None	None	Supports inflammation resolution as prednisone has ended.
Ca-AKG 1,000 mg	None	None	Peak utility: collagen hydroxylation cofactor.
ADAM multi	None. Aspirin and pentoxifylline finished.	Most components	Vit C 250 mg for hydroxylation. Zinc 15 mg for epithelial closure.
CoQ10 200 mg	None	Mostly	Mild antioxidant.
NAD+ CR + Resveratrol	None	None	DNA repair in rapidly dividing cells.
Black Maca	None	Main MOA	Weak murine hair-growth signal only.
Boron 3 mg	Trivial androgen note persists while Problend is paused	Main MOA	Weak matrix-signaling support.
Omega-3	None	None	Resolvins and protectins actively drive inflammation resolution. Net clearly positive from here.
D3/K2	None	None	VDR signaling supports follicle survival into telogen.

Days 15-21 — Early remodeling

Collagen III to I conversion and crosslinking. Follicles enter dormancy. **Needs:** copper and vitamin C for lysyl and prolyl oxidase, clean inflammation resolution.

Supplement (confirmed dose)	Harmful	Neutral	Helpful
NAC 1,200 mg	None	None	Keratin substrate ahead of eventual regrowth.
Creatine 5 g	None	Mostly	Mild energy support.
Glycine 3 g	None	None	Remodeling substrate.
Collagen 14 g + HA + vit C	None	None	Continued remodeling substrate.
Urolithin A 1,000 mg	None	None	Prevents smoldering low-grade inflammation.
Ca-AKG 1,000 mg	None	None	Prolyl and lysyl hydroxylase cofactor for crosslinking.
ADAM multi	None	Most components	Copper 0.5 mg for lysyl oxidase crosslinking. Saw palmetto 160 mg is currently your only 5AR input.
CoQ10 200 mg	None	Mostly	Mild antioxidant.
NAD+ CR + Resveratrol	None	None	Cellular maintenance.
Black Maca	None	Main MOA	Weak
Boron 3 mg	Trivial	Main MOA	Weak
Omega-3	None	None	Net helpful. Resolution phase.
D3/K2	None	None	D3 supports anagen re-entry signaling in dormant follicles.

Synthesis and Action Items

Nothing warrants holding

The bleed concern collapses to a single line item: **100 mg d-alpha tocopherol plus 25 mg grape seed extract in the ADAM multivitamin**, running alongside aspirin, pentoxifylline, and Trintellix. Worth a one-line question to Dr. Sean's office, framed as hold-or-continue for seven days. Everything else is neutral to helpful.

Corrections from the initial analysis

Omega-3 was cleared. Thorne Super EPA at 2 gelcaps delivers 1,390 mg combined EPA+DHA, roughly half the ~3 g threshold where antiplatelet effects become clinically detectable. The earlier recommendation to hold it was wrong.
Resveratrol was cleared. Life Extension's formula contains only 10 mg trans-resveratrol and 5 mg quercetin. Too small to affect platelets.
Creatine edema claim was wrong. Creatine water retention is intracellular and muscle-localized. Post-op scalp and facial swelling is inflammatory extravasation. Oral minoxidil is the dominant fluid-retention input.
Grape seed extract was missed initially. 25 mg in ADAM, proanthocyanidin antiplatelet activity.
Saw palmetto was missed initially. 160 mg in ADAM. With Problend paused, this is currently the only 5-alpha-reductase input in the stack.

Cycling coverage gap

Urolithin A, Ca-AKG, NAD+, and maca are currently weekends-off. The days 8-14 window is peak collagen synthesis, where Ca-AKG and collagen matter most.
Highest-leverage single change: run Ca-AKG straight through days 8-21 rather than weekdays-only. No downside.

Still unverified

CoQ10 brand and form (ubiquinone vs ubiquinol) remains unconfirmed. Changes absorption substantially but nothing in the analysis above. The NOW D3/K2 label specifies only "menaquinone" without distinguishing MK-4 from MK-7; since NOW labels their MK-7 products explicitly as MenaQ7, this is likely MK-4 with a shorter half-life. Minor.

This document summarizes a research conversation and is not medical advice. Supplement and medication decisions during post-operative recovery should be confirmed with Dr. Sean Behnam's office. Generated August 20, 2026.